



Personal Electronic Communication Devices for Mental Health Inpatient Consumers Policy

Scope

Site	Department, division or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	All Mental Health Division (MHD) inpatient areas and other RPBG inpatient areas managing mental health consumers	All Staff

Policy statement

This policy specifies the key principles and processes for consumers, visitors, family and personal support persons (PSPs) regarding the use of personal electronic communication devices (PECDs) within mental health inpatient areas or where a general health patient is detained under the Mental Health Act (MHA) 2014 at the discretion of the psychiatric team.

Links to relevant documents

- [Appendix I: Relevant sections of the Mental Health Act](#)
- Mental Health Act (MHA) 2014

Definition

PECD	Includes devices – i.e. mobile phones, tablet, laptop and any other electronic device that allows personal communication.
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Introduction

Communication with family and friends is an essential element of support and comfort for consumers whilst receiving care as an inpatient which PECDs may provide assistance with.

Many PECDs, particularly mobile phones, commonly have extended functions, such as camera and video recording capability and music players, which could lead to potential breaches of consumer confidentiality, dignity and privacy. In addition, ring tones and/ or music played via mobile phones and/or PECDs can be disturbing and disruptive to others in a therapeutic, recovery orientated inpatient environment.

There are also a number of risks associated with visitors' and consumers' use of PECDs in a ward environment and some of these may contribute to incidents of injury, e.g.:

- Theft and exploitation
- Bullying and harassment of staff and other consumers
- Intrusive, unwanted or obscene phone calls
- Debt associated with incurring large phone bills/data usage (financial harm)
- Use as a weapon
- Problems of illicit photography, videoing and recording
- Health and safety risks associated with equipment not electrically tested to engineering standards
- Mains charging leads and headphone cords used as ligatures.

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Version: 3.1

Endorsed by: Clinical Practice Committee 02/03/2023

Authorised by: Deliver What Matters Most Committee 04/04/2023

Amendment: Executive Director 30/04/2024

Date Compiled: 04/2018

Document Owner: Medical Co-Director MH Division

Revision Due: 04/2026

Policy directives

- Staff are to facilitate the regulation of PECDs within mental health inpatient areas and general wards with patients under the MHA 2014 by direction of the Consultant Psychiatrist, to support and promote:
 - Consumer engagement with staff, treatment, therapeutic programmes/activities and other consumers
 - Positive contact between consumers and friends/family/PSPs
 - Provision of a therapeutic environment with limited distractions
 - Protection of the rights of individuals
 - Protection of individuals from abuse
 - Consumer recovery
 - Confidentiality
 - Consumer privacy and dignity
 - Acceptable standards of behaviour.

Inclusions

All consumers voluntary and involuntary (under the MHA 2014) within inpatient areas and visitors where applicable.

Note: Consumers who are detained under the MHA 2014 may have the use of PECDs restricted. See section, [Consumers use of PECD who are detained under the MHA 2014](#).

EMyU – only Schedule for the use of PECDs

EMyU provides a 7 day therapeutic programme (as per the EMyU Model of Care).

To support consumer engagement in this programme, PECDs are to be stored and charged in the EMyU nursing station.

Consumers are able to collect/return their PECDs from/to the nursing station according to a designated schedule for PECD usage:

- Monday to Friday 1600 – 2200 hours
- Saturday and Sunday 1500 – 2200 hours

Note: These times are negotiable via the Shift Co-ordinator based on ward mix and availability of therapeutic activities.

Responsibilities/procedures

MHD and general wards with consumers under the MHA 2014 under direction of Psychiatry will make every effort to support consumers in making and maintaining contact with family/PSP and friends by telephone, text and email and to enable telephone calls to be made with appropriate privacy, aligning with the [Charter of Mental Health Care Principles \(MHA 2014\)](#).

MHD and general wards will not accept responsibility or liability for loss or damage to any PECD belonging to consumers or visitors and will not accept responsibility or liability for mobile phone bills.

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Responsibilities/procedures cont'd

Admission

On admission, the Allocated Clinician/Nurse Unit Manager (NUM) or delegate/Treating Team is to:

- Advise consumers and visitors of the responsibility for the safekeeping of their PECD and alert them to the risk of theft or loss. Refer to the **R PBG Visitor Policy**
- Refer to any posters/instructions advising consumers about restrictions to use of PECDs in their specific ward/unit areas
- Advise consumers regarding storage of personal property items, where appropriate, as per the **R PBG Patient Property Management Policy/ Potentially Harmful Items Management for Mental Health Patients Standard Operational Procedure (SOP)**
- Advise consumers and visitors/family/PSP of the restrictions placed on the use of PECDs, and how to access loan chargers as set out in this policy
- Ask consumers if they have a PECD and charging equipment in their possession and advise them of the risks of retaining and using these devices whilst in hospital/unit
 - For some consumers it might be appropriate for their parent/guardian/PSP to retain possession of their PECD during the period they are in hospital. These consumers will have their PECD removed and either given to their parent/guardian/PSP or kept in storage. The consumer will be informed of the rationale for restricted use of their PECD
- List the consumers' PECD on the Property Tracking Form/property sheet/appropriate site based form
 - Within **EMyU**, if outside the designated times for PECD access, ensure the PECD is retained for safe keeping and the device is clearly labelled with the name of the owner (affix patient identification label)
- Ensure consumers who wish to retain and access their PECD during their admission sign the formal consumer admission agreement that they will not take images or recordings of anything or anyone and that the PECD will only be used for the purpose of conversation, texts and emails, or social media/entertainment and store this signed agreement in the consumer's medical record
 - If a consumer refuses to agree to this restriction the team will need to decide if the consumer is allowed to retain their PECD and document this decision and the rationale in the consumers integrated notes and the 12C Form
- Where available, request visitors to utilise a locker provided to store all personal belongings prior to entering the ward/service. Where this option is not provided, the visitors are to retain their personal items and are expected to adhere to ward/service rules and **R PBG Visiting Policy**.

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Responsibilities/procedures cont'd

Review and continuation of admission

- Display information about consumers' rights about the use of PECDs, under the MHA 2014, in all relevant areas
- Ensure posters/pamphlets are available on site or in consumer's welcome pack for all consumers and particularly those who retain their PECD for use during their hospitalisation. Consumers' attention will be specifically drawn to the following requirements:
 - Keep the phone on silent setting
 - Take steps to keep PECDs safe and secure at all times
 - Avoid lending PECDs to other consumers
 - Avoid using in communal areas such as lounges and kitchens
 - Abstain from the use of the recording or photography facility
 - No downloading of inappropriate or illicit content
 - Adherence to scheduled times for PECD access - EMyU only (see section, [Schedule for the use of PECD – EMyU](#))
- Ensure signage is displayed in clinical areas (where appropriate) that indicates where PECDs cannot be used such as lounges, dining areas etc.
- Advise consumers that there is a public cordless telephone on wards for their personal use and access to electronic communication via Internet Kiosks, where available.

Consumers use of PECD who are detained under the MHA 2014

Consumers detained under the MHA 2014 have the same rights as voluntary consumers to have contact with family and friends through readily accessible telephone facilities. However, consumers admitted under the MHA 2014 may have restriction(s) placed on the use of PECDs provided there are reasonable clinical grounds to do so and this decision is taken as part of the individual's plan of care and the relevant MHA 2014 documentation is completed (i.e., Form 12C for voluntary/involuntary consumers) and is to be reviewed every 24 hours by a Consultant Psychiatrist.

In accordance with the MHA 2014, the Consultant Psychiatrist is the only person who can direct staff to restrict communication in the least restrictive way possible and with due consideration to alternative options to communicate. For conditions provided in the MHA 2014 to restrict communication, see [Appendix I: Relevant sections of the Mental Health Act](#).

There are recognised potential risks relating to consumers detained under the MHA 2014 who may not have the capacity to safely manage the use of a PECD – e.g. the potential to make frequent and numerous phone calls to police, local politicians, newspapers, media, etc. or to receive inappropriate phone calls which may place themselves or others at risk.¹

Consumers detained under the MHA 2014 who are assessed by the clinical team (Consultant Psychiatrist in general wards) not to have the capacity to manage the identified risks of using a PECD should have the use of their relevant PECD denied for the duration of the risk. This should be discussed with the individual, their parent/guardian/PSP, reviewed every 24 hours with the clinical team (Consultant Psychiatrist in general wards) and the consumer and documented in the consumer's integrated notes.

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Responsibilities/procedures cont'd

Misuse /breaches of consumer's use of the PECD

Potential or actual breaches/misuse of a PECD may include the following:

- Injury to self and/or others
- Unlawful use e.g.
 - Audio and/or visual recordings without the consent of all parties captured in the recording (this also contravenes the Surveillance Devices Act 1998)
 - Phone use/calls (e.g. stalking, harassment, threats of violence)
 - Access to social media is non-therapeutic – e.g. cyber bullying including bilateral bullying
 - Social media disclosure of staff or other consumers' identity details
 - Downloading of prohibited and/or offensive content e.g.
 - Depictions of sexual activity, high impact violence, offensive sexual practices, detailed instruction of crime¹.

Should a concern arise at any time regarding a consumer's misuse of their PECD a risk assessment is to be undertaken which may result in the PECD being removed from the consumer. Where this action is taken, a clear explanation will be given to the consumer and their parent/guardian/PSP (where appropriate) and documented in the consumer's integrated notes.

If a decision is made to remove a consumer's PECD, the PECD can either be given to the parent guardian/PSP for safekeeping or be placed in secured storage within the inpatient area until the consumer is granted leave or is discharged from the ward.

Consumers who are found to be using and/or have used their PECD to record or take pictures of other consumers, and/or staff, and/or situations on MHD or general wards with patients under the MHA 2014 premises will be asked to delete these images and any 'cloud storage' and/ or image sharing platform. If the consumer refuses to do so MHD staff will retrieve the PECD and delete the images if necessary.

Should staff discover that prohibited illicit or offensive content has been downloaded the treating team is to be advised to consider if it is appropriate for the police to be informed.

Following any such events, the PECD will be removed from the consumer and placed in secured storage within the inpatient area until a discussion at the next clinical/ multidisciplinary (MDT) review takes place. The reasons for removal and subsequent clinical decisions will be discussed with the consumer and should be clearly documented in the consumer's integrated notes and may form part of their management plan.

In the event of a visitor refusing to respect these restrictions the visitor will be asked to leave the clinical area and the multi-disciplinary team may decide to refuse entry to future visits.

In the event of a consumer misusing a PECD despite clear communication about appropriate use whilst admitted, the Allocated Clinician/consumer's treating team are to:

- Confiscate and store or hand the PECD to the consumer's parent/guardian/PSP for safekeeping/secure area
- Involve police if there is clear evidence of unlawful use

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Amendment Executive Director 30/04/2024

Responsibilities/procedures - Misuse /Breaches of consumer's use of the PECD cont'd.

- As much as possible, enforce restrictions in a recovery focussed and least restrictive manner¹
- Incorporate any enforced restrictions and associated risk management strategies related to a consumer's use of PECD into the Psychiatric Services Online Information System (PSOLIS) Client Management Plan, Comprehensive Care Plan, What Matters Most Record and where possible, involve the consumer and family/guardian/PSP. Refer to the **RPBG Comprehensive Care SOP**
- Employ strategies to manage consumers who are at risk of becoming violent or aggressive to others (e.g. other consumers and/or staff and/or visitors), including de-escalation tactics. Refer to the **Department of Health Clinical Care of People With Mental Health Problems Who May Be At Risk of Becoming Violent or Aggressive Policy/ RPBG Restraint and Seclusion Minimisation and Safe Management (for Mental Health Areas) Clinical Practice Standard (CPS)**
- Depending on the risks (e.g. high risk of violence/aggression for phone removal), consider contacting Security and/or police, where appropriate, to assist, in order to promote the safety of all consumers, staff and visitors. Refer to the **EMHS Security Policy**
- Monitor and cease restrictions as soon as the indication for restriction no longer applies
- Offer supportive counselling about conversations with a consumer relating to any intended decision to restrict PECD use and after the implementation of PECD restriction, if appropriate²
- Document in the consumer's integrated notes all rationale, management plans and outcomes regarding a consumer's use of PECD.

Charging of PECD

All MHD and general ward electronic equipment is required to be tested by RPBG engineering staff to fulfil safety requirements. Consumers may bring their own charging equipment into hospital. Charging equipment is available on some wards which are compatible with PECDs and are regularly tested by the MHD.

If required, personal charging equipment may be put in storage or given to a parent/guardian/ PSP for removal and safekeeping, as per the **Patient Property Management Policy/ Potentially Harmful Items Management for Mental Health Patients SOP**. Staff are to ask the consumer to switch off their PECD, prior to charging. If the consumer refuses to do so, the PECD will not be charged due to security purposes and this will be discussed with the consumer and should be documented in the consumer's integrated notes.

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Compliance monitoring

It is the responsibility of all staff working within the MHD and general wards with patients under the MHA 2014 to comply with this Policy.

Compliance will be monitored as part of the:

- Clinical Incident Management (CIM) system process and/or Combined Hazard or Incident Report (CHoIR) form related to PECD
- Compliments/Complaints Management System.

It is the responsibility of the NUM/delegate and Shift Co-ordinator to ensure that compliance with this Policy is monitored.

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We acknowledge the following previous site endorsed work and/or contributors used to compile this document.

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Related policies, practice standard, clinical guidelines

[RPBG Policy Hub](#)

- Comprehensive Care SOP
- Admission, Discharge and Transfer SOP
- East Metropolitan Youth Unit (EMyU) Admission and Discharge Procedure SOP
- Patient Property Management Policy
- Visiting Policy
- Restraint and Seclusion Minimisation and Safe Management (for Mental Health Areas) CPS
- Potentially Harmful Items Management for Mental Health Patients SOP

[EMHS Policy Hub](#)

- Rights of Patients in Mental Health Services Policy
- Prevention and Management of Workplace Aggression and Violence Policy
- Security Policy

[Department of Health Policy Hub](#)

- [Safety Planning for Mental Health Consumers Policy](#)
- [Principles and Best Practice for the Care of People Who May Be Suicidal](#)
- [Safety Planning Procedures for Mental Health Consumers](#)
- [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)
- State-wide Standardised Clinical Documentation (SSCD)
- Clinical Incident Management Policy

Other

- Mental Health Act 2014
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)
- WA Chief Psychiatrist's Standards for Clinical Care 2015
- State Records Act 2000
- Surveillance Devices Act 1998 (WA)

Related National Standards

ACSQHC NSQHS Standards 2nd Edition (2021); Standard 1: Clinical Governance; Standard 2: Partnering with Consumers

NMHS Standards (2010): Standard 1: Rights and responsibilities; Standard 2: Safety; Standard 3: Consumer and carer participation; Standard 6: Consumers; Standard 10: delivery of care

Australian Charter of Healthcare Rights Guiding Principles: Safety and Respect.

Charter of Mental Health Care Principles MHA 2014 Principles 1, 2 & 4.

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Appendix I: Relevant sections of the Mental Health Act 2014

Section (Part 3, Division 2)	Provisions
Communicate (s. 14)	In relation to an inpatient, means: • sending from, or receiving at, a designated mental health service uncensored private communication which may include communication by letter, telephone or electronic means; or • receiving visitors at a designated mental health service at reasonable times, including the Australian legal practitioner or nominated person of the inpatient.
Right to communicate (s.15)	• An inpatient has a right to communicate lawfully with any person. • An inpatient has a right to communicate lawfully with any person for the purpose of seeking legal advice or legal representation. • Members of staff of a designated mental health service must ensure that reasonable steps are taken to assist an inpatient to communicate lawfully with any person.
Restrictions on right to communicate (s.16)	• An authorised psychiatrist may in writing direct staff at a designated mental health service to restrict an inpatient's right to communicate if the authorised psychiatrist is satisfied that the restriction is reasonably necessary to protect the health, safety and wellbeing of the inpatient or of another person. • A direction cannot be given which restricts an inpatient's right to communicate with: • a legal representative; or • the chief psychiatrist; or • the Mental Health Complaints Commissioner; or • the Mental Health tribunal; or • a community visitor; or • a prescribed person or body* *The Independent Mental Health Advocacy Service is a prescribed body for the purposes of s.16 (2) (f) of the Act. • An authorised psychiatrist must ensure that if he or she directs that the inpatient's right to communicate is restricted, those restrictions are the least restrictive possible to protect the health, safety and wellbeing of the inpatient or of another person.
Persons to be notified of restriction on inpatient's right to communicate (s. 17)	• An authorised psychiatrist who makes a direction to restrict an inpatient's right to communicate must ensure that reasonable steps are taken to inform the inpatient and the following persons in relation to the inpatient about the restriction, and the reason for it; • the nominated person • a guardian • a carer • a parent (if the inpatient is under the age of 16 years) • the secretary to the Department of Health and Human Services, if the inpatient; • is the subject of family reunification order or a care by Secretary order.
Restriction on right to communicate to be monitored regularly (s.18)	• An authorised psychiatrist who makes a direction to restrict an inpatient's right to communicate must review his or her decision on a regular basis to determine whether the restriction needs to be continued. • If the authorised psychiatrist is satisfied that it is no longer necessary to restrict the inpatient's right to communicate, the authorised psychiatrist must immediately cease the restriction.

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